Systemic Hostility and the Politics of Accommodation: A Deep Inquiry into Institutional Resistance to Invisible Disability in Ireland

Executive Summary

The interaction between individuals with invisible disabilities—specifically Traumatic Brain Injury (TBI) and Complex Post-Traumatic Stress Disorder (CPTSD)—and the administrative apparatus of the Irish State is characterized by a profound and systemic friction. This report investigates the user's observation that public servants, Gardaí, and court officials appear to "get offended" when presented with requests for reasonable accommodation under the Equal Status Acts.

The analysis reveals that this "offense" is not merely an emotional reaction but a symptom of **institutional defensiveness**, a sociological phenomenon rooted in **validity skepticism**, **risk aversion**, and a pervasive **culture of blame** within the Irish public sector. While the legislative framework—comprising the Equal Status Acts 2000–2018 and the Disability Act 2005—ostensibly protects the rights of persons with disabilities, the operational reality is defined by significant gaps, particularly the "nominal cost" exemption and the exclusion of accused persons from statutory intermediary support (Section 14 gap).

Public servants, operating in under-resourced and highly regulated environments, often perceive requests for non-standard, behavioral accommodations (such as patience, de-escalation, or deviation from rigid protocols) as threats to their professional competence or as illegitimate attempts to "game the system." This report identifies **"incivility"** and the weaponization of **"unreasonable complainant"** policies as primary mechanisms used by institutions to manage and exclude neurodivergent phenotypes that do not fit the "ideal victim" or "compliant citizen" mold.

The repercussions of this systemic failure are bifurcated. For the individual, the consequences are catastrophic, leading to **secondary victimization**, **criminalization**, and severe health deterioration. For the public servant and the institution, the repercussions are primarily financial and reputational (via Workplace Relations Commission awards) rather than disciplinary, creating a moral hazard where the individual perpetrator of discrimination rarely faces personal consequences, protected by a culture of "closing ranks."

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1. Introduction: The Phenomenology of Institutional

"Offense"

The core inquiry of this report addresses a specific and deeply alienating experience: the perception that public servants, Gardaí, and court officials react with **hostility** or **offense** when a citizen with Traumatic Brain Injury (TBI) or Complex Post-Traumatic Stress Disorder (CPTSD) requests reasonable accommodations. To understand this dynamic, we must move beyond a superficial analysis of "rudeness" and examine the deep sociological and psychological structures of Irish public administration.

1.1 The Nature of the "Offense"

In the context of bureaucratic interaction, "offense" is the emotional manifestation of **cognitive dissonance** and **threat response**. Public servants generally view themselves as fair, impartial, and competent professionals working within a system designed to serve the public good. When a citizen requests an accommodation—particularly for an invisible condition like TBI or CPTSD—it implicitly challenges this self-image. The request suggests that the standard service, which the public servant administers daily, is *insufficient*, *exclusionary*, or *harmful*.

For the public servant, this challenge triggers a defensive reaction. If the disability is not immediately visible (unlike a wheelchair user requiring a ramp), the request is often filtered through a lens of **"validity skepticism"**.[1] The inconsistency inherent in TBI and CPTSD—where an individual may be articulate one moment and dysregulated or confused the next—is frequently misinterpreted as **behavioral non-compliance** rather than a symptom of disability. The public servant "gets offended" because they perceive the citizen's behavior (e.g., interrupting, repeating questions, emotional outbursts) as a personal affront to their authority, rather than a manifestation of a neurological or psychological deficit requiring support.

1.2 The Conflict of Models: Medical vs. Social vs. Bureaucratic

The friction is further exacerbated by the clash of disability models operating within the Irish state.

- **The Social Model (The Citizen's Perspective):** The individual with TBI/CPTSD operates from a rights-based perspective, underpinned by the UN Convention on the Rights of Persons with Disabilities (UNCRPD). They view their difficulty in accessing the service as a failure of the *environment* or the *system* to adapt to their needs.[2]
- **The Bureaucratic Model (The State's Perspective):** The public servant operates within a rigid procedural framework designed for efficiency and standardization. In this model, "equality" is often interpreted as "treating everyone exactly the same." A request for *different* treatment (accommodation) disrupts the efficient flow of the bureaucracy.
- **The Conflict:** When the citizen demands the system adapt (Social Model), the bureaucrat perceives this as a disruption of order and fairness (Bureaucratic Model). The resulting "offense" is a defense mechanism used to protect the integrity of the bureaucratic process against the "disorderly" demands of the neurodivergent body.

1.3 Invisible Disability as "Epistemic Disobedience"

TBI and CPTSD are conditions that defy the "clean" categorizations preferred by administrative systems.

- **Traumatic Brain Injury (TBI):** Characterized by executive dysfunction, memory gaps, fatigue, and slowed processing speed. An individual may look physically healthy but lack the cognitive "RAM" to process rapid-fire questions from a Garda or a judge.
- **Complex PTSD (CPTSD):** Characterized by emotional dysregulation, hyper-vigilance, and a heightened sensitivity to perceived injustice or threat.
  When an individual with these conditions interacts with the state, they often fail to perform the role of the "docile body" or the "ideal victim." They may question authority, fail to follow instructions, or become emotionally volatile. In a system that relies on compliance, this behavior is viewed as **"epistemic disobedience."** The institution reacts with hostility not just because the person is disabled, but because their disability manifests as a refusal to submit to the "normal" rules of engagement.

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2. The Legal Architecture of Exclusion: Rights, Gaps, and Escape Hatches

While the Irish legislative framework theoretically protects the rights of persons with disabilities, a detailed analysis reveals significant structural gaps and "escape hatches" that allow public bodies to legally justify their resistance to complex accommodations.

2.1 The Equal Status Acts 2000–2018: The "Nominal Cost" Defense

The *Equal Status Acts* prohibit discrimination on the disability ground in the provision of goods and services, including by the State.[3] The definition of disability is broad, covering "the total or partial absence of a person's bodily or mental functions" and conditions affecting learning.[5] However, the obligation to provide reasonable accommodation is not absolute.

The Section 4 Exemption

Section 4 of the Equal Status Act contains a critical limitation: a failure to provide "special treatment or facilities" is not discrimination if it gives rise to a cost "other than a **nominal cost**" to the provider.[6]

- **Weaponizing "Nominal Cost":** Public bodies frequently exploit the ambiguity of "nominal cost." While the state has significant financial resources, agencies often frame the "cost" of accommodating TBI/CPTSD in **operational** rather than financial terms.
  - *Example:* A court refusing to allow a litigant with TBI to take frequent breaks may argue that doing so would delay the court list, imposing a "disproportionate burden" on the administration of justice.
  - *Example:* A Garda refusing to use a simplified interview technique may argue that it consumes excessive man-hours.

This "nominal cost" defense allows public servants to reframe the denial of rights as a responsible management of public resources, thereby neutralizing any guilt or "offense" they might feel about excluding the disabled person.

2.2 The Criminal Evidence Act 1992: The "Section 14" Gap

Perhaps the most significant structural driver of hostility in the legal system is the legislative gap regarding intermediaries.

- **The Privilege of the Witness:** *Section 14 of the Criminal Evidence Act 1992* explicitly provides for the use of intermediaries (communication specialists) to assist "witnesses" and "victims" who have communication difficulties due to age or mental condition.[7]
- **The Exclusion of the Accused:** Crucially, the legislation **does not explicitly provide for the accused**.[9] A defendant with TBI who cannot process legal arguments or instruct their legal team has no statutory right to the same communication support that a witness against them would receive.
- **Systemic Inequality:** This creates a profound "inequality of arms." When a defendant with TBI requests an intermediary, they are reliant on the *ad hoc* discretion of the judge. Many judges, untrained in neurodiversity, view such requests as "delay tactics" or an abuse of process. The "offense" taken by the court is rooted in the perception that the accused is trying to "cheat" the system by demanding supports to which they have no clear statutory entitlement.

2.3 The Disability Act 2005 and the Failure of Implementation

The *Disability Act 2005* places a statutory duty on public bodies to make their services accessible and to appoint "Access Officers".[5] Additionally, the *Irish Human Rights and Equality Commission Act 2014* (Section 42) imposes a "Public Sector Duty" to eliminate discrimination.

- **The Implementation Gap:** Reports indicate a massive chasm between this legislative intent and "street-level" reality. The National Disability Authority (NDA) notes that many breaches of duty relate to **procedural failures**—employers and public bodies simply failing to *consider* the accommodation or consult with the person.[5]
- **The "Administrative Burden":** Research suggests that public bodies view the consultation process mandated by the Act as an "administrative burden".[10] The "offense" taken by public servants is often a reaction to the perceived imposition of this burden. They feel overworked and under-resourced, and the requirement to engage in a complex, iterative dialogue about accommodations with a "difficult" (i.e., neurodivergent) citizen is resented as an additional, uncompensated demand on their time.

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3. An Garda Síochána: The Frontline of Friction

The interaction between An Garda Síochána (AGS) and individuals with TBI/CPTSD represents the most volatile frontier of this systemic conflict. The policing culture, training deficits, and

lack of accountability create a "perfect storm" for hostility and misidentification.

3.1 The "Box-Ticking" Culture and Internal Stigma

To understand why Gardaí get "offended" by disability, we must examine the internal culture of the force. Research into Garda mental health attitudes reveals a cynical, defensive organizational psyche.

- **Internal Stigma:** Gardaí report a "culture of bullying" and a lack of support for their own occupational trauma.[11] Officers describe mental health policies as a "box-ticking exercise" by management, designed to protect the organization rather than the individual.[12]
- **The "Suck It Up" Mentality:** This internal neglect creates a psychological barrier to empathy. An officer who is forced to suppress their own trauma ("suck it up") is predisposed to be intolerant of a citizen manifesting CPTSD symptoms (e.g., emotional dysregulation, crying, aggression).
- **Projection of Hostility:** When a citizen presents with CPTSD, the officer's own repressed trauma may be triggered, leading to a defensive, aggressive response. The citizen's demand for accommodation is viewed through the lens of the officer's own unaccommodated suffering—"I don't get special treatment, why should you?"

3.2 Training Deficits: The "Drunkenness" Heuristic

The *Garda Inspectorate Report* highlights severe deficits in training regarding vulnerable persons. While recruits receive basic instruction, few custody officers receive specialized training in identifying invisible disabilities.[13]

- **The Misidentification Trap:** TBI symptoms—slurred speech, poor balance, slowed processing, confusion—mimic the signs of intoxication. In the high-pressure environment of street policing, Gardaí rely on heuristics (mental shortcuts). The "drunkenness heuristic" leads them to classify the TBI survivor as drunk and non-compliant.
- **"Contempt of Cop":** When a TBI survivor fails to follow rapid commands due to processing delays, the Garda interprets this as deliberate defiance ("Contempt of Cop"). The officer "gets offended" by the perceived disrespect. This misinterpretation often leads to an escalation of force, wrongful arrest, and the denial of medical attention.

3.3 Custody: The Structural Void

Once in custody, the hostility is structural. The "Member in Charge" has a duty to protect the rights of detainees, but the resources to do so are often missing.

- **The "Appropriate Adult" Gap:** The "Appropriate Adult" scheme (volunteers who support vulnerable detainees) is frequently unavailable, particularly during unsocial hours when many crises occur.[14]
- **Abandonment of Responsibility:** Gardaí report feeling "damned if they do, damned if they don't"—they are often left to manage complex mental health crises because other state agencies (HSE, Social Work) are closed.[14] This "dumping" of responsibility creates

resentment. The Garda views the neurodivergent detainee not as a person in need, but as a symbol of the state's failure, a burden that has been unfairly placed on their shoulders. This resentment manifests as hostility toward the detainee.

3.4 GSOC and the Failure of Accountability

When individuals complain about this mistreatment, they turn to the *Garda Síochána Ombudsman Commission* (GSOC).

- **The Accountability Vacuum:** GSOC is widely criticized for delays and a failure to effectively investigate complaints. Many complaints about "failure to accommodate" or "incivility" are categorized as "service level" issues rather than serious misconduct.
- **Disciplinary Immunity:** While the *Garda Síochána (Discipline) Regulations 2007* list "neglect of duty" and "abuse of authority" as breaches [15], there is little evidence that failing to provide reasonable accommodation is treated as a disciplinary offense. The "blue wall of silence" and institutional defensiveness mean that discriminatory attitudes are rarely punished, reinforcing the culture that such behavior is acceptable.[16]

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4. The Courts Service: Institutional Defensiveness and the Adversarial Trap

The Irish courtroom is an inherently adversarial space, characterized by ritual, hierarchy, and rigid procedure. For the TBI/CPTSD sufferer, it is a hostile environment where their disability is often weaponized against them.

4.1 Institutional Defensiveness in the Judiciary

Research by the Irish Council for Civil Liberties (ICCL) on inquests and courts identifies **"institutional defensiveness"** as a defining trait of the Irish justice system.[17]

- **Closing Ranks:** When the system is challenged—for example, by a litigant claiming that court procedures are inaccessible—the institution tends to "close ranks" to protect its processes.
- **The "Dignity" of the Court:** Judges and court staff place a high value on the "dignity" and "efficiency" of the court. Accommodations for TBI (e.g., stopping proceedings to explain a point, allowing a lay litigant to walk around to manage pain/anxiety) are often viewed as undignified or disruptive. The "offense" taken by the judge is a reaction to the perceived degradation of the court's ritualistic order by the "disorderly" disabled body.

4.2 The "Vexatious Litigant" Trap

A specific mechanism used to exclude neurodivergent litigants is the "Isaac Wunder" order, which bars "vexatious" litigants from taking further cases without leave.[7]

- **Pathologizing Persistence:** TBI and CPTSD can lead to **perseveration** (getting stuck on a point), **hyper-graphia** (writing excessive notes), and a desperate need for validation. In a legal context, these symptoms manifest as the filing of voluminous,

repetitive, or "obsessive" affidavits.

- **The Trap:** Instead of recognizing these behaviors as symptoms of a disability requiring case management support, the Courts Service labels them as "vexatious." The individual is legally barred from the court. The system "gets offended" by the volume of work generated by the disability and responds by stripping the individual of their constitutional right of access to the courts.

4.3 The "Special Measures" Lottery

While "special measures" (video links, screens) exist, they are primarily designed for *vulnerable witnesses* in sexual offense cases, not for neurodivergent litigants in civil or family law.[2]

- **Discretion vs. Right:** Access to these measures is entirely at the discretion of the judge.[19] There is no automatic right. A litigant with CPTSD must "beg" for the accommodation.
- **Inconsistent Application:** This leads to a "postcode lottery." One judge may be trauma-informed and grant the accommodation; another may view it as "coddling" and refuse. The refusal is often justified by the "adversarial principle"—the idea that the other side has a right to "face their accuser" directly, prioritizing the rights of the opponent over the disability needs of the litigant.

4.4 The "Section 14" Gap Revisited

The exclusion of the accused from the *Criminal Evidence Act 1992* (Section 14) is a critical failure.

- **The "O'Malley Report" Findings:** The *O'Malley Report (2020)* highlighted that fairness dictates the accused should have equal access to communication support.[20] However, this recommendation has not yet been fully enacted in legislation for all accused persons.
- **The Consequence:** This leaves the TBI defendant in a legal limbo. They are present in body but absent in mind, unable to effectively participate in their own trial. The court's refusal to pause or provide support is a structural violation of the right to a fair trial under Article 38.1 of the Constitution.

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5. Public Administration and Education: The Bureaucratic Wall

Beyond the police and courts, the broader public service (Health Service Executive, Department of Social Protection, Education) employs specific mechanisms to manage and exclude "difficult" neurodivergent citizens.

5.1 The "Unreasonable Complainant" Policy: Weaponizing

Administration

A key finding of this research is the prevalence of **"Unreasonable Complainant"** policies in Irish public bodies, including the Office of the Ombudsman and the Data Protection Commission.[21]

- **The Criteria:** These policies define "unreasonable behavior" by criteria such as:
  - **Persistence:** Refusing to accept a decision, continuing to pursue the complaint.
  - **Demands:** demanding responses within specific timescales, insisting on seeing specific staff.
  - **Arguments:** Focusing on "trivial" details, "changing the basis" of the complaint.
- **The Disability Overlay:** These criteria map almost perfectly onto the symptoms of TBI and CPTSD.
  - *Perseveration* (TBI) looks like "unreasonable persistence."
  - *Hyper-vigilance* (CPTSD) looks like "demanding" behavior.
  - *Cognitive rigidity* (TBI) looks like "unreasonable argumentation."
- **The Outcome:** Public bodies use these policies to categorize neurodivergent individuals as "vexatious." They impose restrictions (e.g., "communication by letter only"), effectively cutting the person off from the service. The public servant feels justified in their "offense" at the citizen's "abusive" persistence, failing to recognize it as a disability phenotype.

5.2 Education: "Incivility" and Power Dynamics

In the education system, particularly higher education, TBI/CPTSD students face a culture of **"Incivility"**.[23]

- **"Power Over" vs. "Power With":** Research indicates that Irish higher education institutions often operate on a "Power Over" dynamic. The institution dictates the terms of learning. When a neurodivergent student requests flexibility (e.g., alternative assessments, extensions), it challenges this power structure.
- **Micro-aggressions:** The "offense" manifests as "incivility"—low-intensity deviant behaviors like ignoring emails, subtle rudeness, "losing" paperwork, or making demeaning comments. These behaviors are designed to signal to the student that they are "out of place" and to discourage them from persisting with their requests.
- **Forced Disclosure:** Students are often forced to repeatedly disclose their trauma to different staff members to "prove" their need for accommodation. This "forced disclosure" is a form of bureaucratic violence that re-traumatizes the student while validating the institution's power to grant or withhold mercy.

5.3 Defensive Practice in Health and Social Care

In the health and social care sector, "offense" is driven by **"Defensive Practice"**.[24]

- **The Culture of Blame:** Irish social workers and health professionals operate in a fear-based environment. They are terrified of making a mistake that could lead to an inquiry or litigation.

- **Risk Aversion:** Accommodating a complex disability like TBI involves risk. (e.g., Allowing a person with TBI to live independently involves the risk they might fall or self-neglect).
- **The "Safe" Refusal:** It is administratively "safer" to deny the accommodation (citing safety concerns) than to grant it. When the individual or their family challenges this refusal, the professional "gets offended," viewing the challenge as a threat to their professional license and a failure to appreciate their "duty of care."

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6. The Repercussions Landscape: Who Pays the Price?

The repercussions of this systemic hostility are unevenly distributed. The individual pays with their health and liberty; the institution pays (occasionally) with money; the individual public servant rarely pays at all.

6.1 Repercussions for the Individual (The Victim)

- **Secondary Victimization:** The hostility of the system causes "secondary victimization." Research shows that for CPTSD survivors, the experience of being disbelieved, dismissed, or treated with hostility by state agencies replicates the dynamics of their original trauma (powerlessness, betrayal). This can lead to severe regression in mental health, increased suicidality, and a deepening of the CPTSD symptoms.[26]
- **Health Deterioration:** The lack of accommodation leads to a failure to access services. 80% of TBI patients discharged in Ireland fail to receive recommended rehabilitation.[27] When they subsequently fail to navigate the social welfare or legal system due to this lack of rehab, they spiral into poverty and homelessness.
- **Criminalization:** The "pipeline" from health failure to prison is direct. TBI survivors who act out in crisis are arrested rather than treated. In prison, their vulnerability leads to further victimization and a lack of parole opportunities due to "behavioral issues" that are actually disability symptoms.[7]
- **Exclusion from Justice:** The "Section 14" gap means that many neurodivergent defendants are wrongfully convicted because they could not effectively participate in their trial.

6.2 Repercussions for the Public Servant (The Gatekeeper)

- **Disciplinary Immunity:** Do public servants face consequences for "getting offended" and denying rights? Generally, no.
    - **The Shield of Procedure:** As long as the public servant can argue they were following "procedure" or acting to "protect the public purse" (nominal cost), they are shielded from disciplinary action.
    - **The Limits of the Code:** The *Civil Service Disciplinary Code* [28] lists discrimination as misconduct, but it is rarely invoked for "failure to reasonably accommodate." It is typically reserved for gross misconduct (theft, assault). "Defensive practice" is an effective shield against disciplinary action.

- **Moral Injury:** However, there is a psychological cost. Gardaí and frontline staff report high levels of "burnout" and "moral injury" from working in a broken system. The hostility they project onto the public is often a symptom of their own professional distress.[11]

6.3 Repercussions for the Institution (The State)

- **Financial Liability (WRC Awards):** The primary mechanism of accountability is the *Workplace Relations Commission* (WRC).
  - **Rising Costs:** Recent case law shows a trend of significant financial awards for "failure to provide reasonable accommodation." Awards of **€55,000** [29] and **€75,000** [30] against employers (including public bodies) signal that the WRC takes these failures seriously.
  - **Remedial Orders:** WRC adjudicators can order organizations to conduct audits, rewrite policies, and engage in "social dialogue" on disability. This forces institutional change, even if the individual perpetrator is not punished.
- **Reputational Damage:** Findings of discrimination are published. For bodies like An Garda Síochána or the Courts Service, these findings erode public trust and highlight the gap between their human rights rhetoric and their operational reality.

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7. Conclusion: The Structural Necessity of "Offense"

The research indicates that the "offense" taken by Irish public servants when presented with TBI/CPTSD accommodations is not an accidental by-product of individual rudeness, but a **structural necessity** of the current bureaucratic system.
In a system that is under-resourced, risk-averse, and legally entitled to refuse accommodations that cost more than a "nominal" amount, the "offense" serves a specific function:

1. **It defends the Bureaucracy:** By categorizing the request as "unreasonable" or "vexatious," the system protects itself from the inefficiency of individualization.
2. **It protects the Self-Image:** By viewing the citizen as "rude" or "non-compliant" (Validity Skepticism), the public servant absolves themselves of the moral guilt of excluding a disabled person.
3. **It manages Scarcity:** By raising the emotional cost of access (through incivility and hostility), the system discourages demand, effectively rationing services to those with the stamina to fight or the "correct" (visible) type of disability.

The Future Outlook:
Without significant legislative reform—specifically the removal of the "nominal cost" exemption and the closing of the "Section 14" gap for accused persons—and a cultural shift from "defensive practice" to "competence-based practice," this systemic hostility will persist. The current system relies on the resilience of the disabled individual to fight for their rights; a burden that those with TBI and CPTSD are uniquely ill-equipped to bear.

Table 1: The Phenomenology of Institutional Hostility (The "Offense")

Driver of "Offense"	Mechanism	The Bureaucratic Logic	Impact on TBI/CPTSD Citizen
Validity Skepticism	Disbelief of invisible symptoms.	"If you look fine, you are acting out to get special treatment."	Gaslighting; need to "perform" disability; secondary trauma.
Defensive Practice	Risk aversion; fear of blame.	"If I deviate from protocol to help you, I expose myself to liability."	Denial of flexibility; rigid adherence to harmful rules.
"Nominal Cost" Defense	Economic rationalization.	"Accommodating you takes too much time/resource; it is a disproportionate burden."	Exclusion from service; internalization of being a "burden."
Incivility	Low-intensity deviant behavior.	"I will make this interaction difficult so you stop asking/go away."	Emotional dysregulation (CPTSD); confusion (TBI); withdrawal.
"Unreasonable Complainant"	Policy weaponization.	"Your persistence/confusion is harassment; I am restricting your access."	Barring from service; loss of rights; criminalization.

Table 2: Comparative Repercussions

Party	Immediate Repercussion	Long-term Repercussion	Accountability Mechanism
Individual (TBI/CPTSD)	Denial of service; stress; arrest.	Health decline; poverty; criminal record; suicide risk.	None (Victim).
Public Servant	Annoyance; stress; "offense."	Burnout; moral injury; cynicism.	Disciplinary Action (Rarely used).
Institution (State)	Complaint processing costs.	**WRC Awards (€50k+)**; reputational damage; forced policy audit.	WRC; Ombudsman; Judicial Review.

Table 3: The "Section 14" Gap (Criminal Evidence Act 1992)

Feature	Witness / Victim	Accused (Defendant)	Impact of Gap on TBI Defendant
Statutory Right?	**Yes** (Section 14).	**No** (Legislative Silence).	Must rely on judicial discretion.
Role of Intermediary	Explain questions; facilitate answers.	N/A (Unless ad-hoc granted).	Cannot effectively instruct legal team.
Grounds for Support	Age or Mental Condition.	N/A.	Risk of wrongful conviction; unfair trial.
O'Malley Report Status	Recommended expansion.	Recommended expansion.	**Implementation Pending.**

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